

TO: David.Chen@primaryhealth.com

FROM: B.Carter@med.unicancer.org

DATE: 15-Mar-2024

SUBJECT: Update re: Paul Clark (DOB 04-Aug-1978, MRN: AML019) - AML in Remission

Hi David,

Hope you are well.

I'm writing with a very positive update regarding our mutual patient, Paul Clark. As you know, we diagnosed him with favorable-risk Acute Myeloid Leukemia (CEBPA mutated) back in April of last year. It's been a long road for him, but I'm thrilled to report that he has successfully completed his entire chemotherapy protocol and is now in a confirmed molecular complete remission.

To recap his journey:

Paul was admitted on April 7, 2023, and began standard "7+3" induction chemotherapy two days later. His induction phase was relatively standard, though he did struggle with significant Grade 3 mucositis and required TPN for about 10 days. However, his day 28 bone marrow biopsy showed an excellent response with no evidence of leukemia.

Following this, he completed three cycles of consolidation therapy with high-dose cytarabine (HiDAC), with the final cycle completed in October 2023. He tolerated consolidation much better than induction.

We just saw him for his 11-month post-treatment follow-up. He looks fantastic. His energy has returned, he's regained the 20 pounds he lost, and he's planning to return to his work as an architect next month. His most recent bone marrow biopsy from last week remains negative for disease, and importantly, his MRD (minimal residual disease) testing by multi-parameter flow cytometry is also negative.

His labs from yesterday are pristine: WBC 5.8, Hgb 14.2, Plt 278.

At this stage, we transition him to a surveillance phase. My plan is:

- Monthly CBC checks for the next year (I've given him the lab slips).
- Follow-up with me in clinic every 3 months.
- Repeat surveillance bone marrow biopsy at the 1-year and 2-year mark post-treatment.

I have asked him to re-establish care with you for all non-hematologic matters. His primary issue that I know he's discussed with you in the past is his anxiety, which was understandably heightened throughout this process. He may benefit from your continued guidance there. Otherwise, he has no active medical issues.

This is truly the best possible outcome for a patient with AML. While we can never say "cured," his favorable genetics and deep molecular response give him an excellent long-term prognosis.

Please feel free to reach out with any questions at all. It's been a pleasure collaborating with you on his care.

Best regards,

Ben

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